

Policy on Planned Facilities and Environment-Friendly Measures

«Hospital Name»

Document control

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Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Engineering In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organisation's environment and facilities operate in a planned manner and promotes environment-friendly measures.

1. Purpose

This policy sets out how «Hospital Name» keeps its environment and facilities operating in a planned, environment-friendly manner. It covers:

- Ensuring facilities and space match the services provided
- Keeping as-built drawings current
- Maintaining comprehensible signage
- Providing potable water and electricity around the clock
- Providing and regularly testing backup sources for both

This policy covers planned facilities and environment-friendly measures specifically. Related duties — like patient assessment, clinical care, medication management, patient rights, infection control, quality and safety monitoring, or hospital governance — are covered in the hospital's other policies, not repeated here.

Words marked like this are defaults. A blank marked _____ must be filled before issue.

2. Scope

This policy applies to «Hospital Name» staff involved in facilities, utilities, medical gases, and fire or non-fire emergencies, including:

- Engineering, biomedical, nursing and departmental leaders
- The Engineering In-Charge
- The Medical Superintendent
- Departmental leaders
- The Quality Coordinator

This policy covers planned facilities and environment-friendly measures specifically. Related duties are covered in the hospital's other policies, not repeated here, including:

- Patient assessment
- Clinical care
- Medication management
- Patient rights
- Infection control
- Quality and safety monitoring
- Hospital governance

3. Policy standards

Facilities and space at «Hospital Name» match the services offered. As-built and updated drawings are maintained by a named custodian. Internal and external signage is in a form patients, families and the community can understand, and meets statutory posting requirements. Potable water and electricity are available round the clock with tested backup sources for any failure.

Staff follow written guidance and keep the records listed in the traceability table.

4. Non-negotiable rules

1. Do not operate a clinical service without matching facility space; any service without a corresponding documented space is not available from this hospital for accreditation purposes.
2. Do not let the as-built and updated drawing set become incomplete or without a named custodian; a drawing that does not reflect the current facility is not an as-built drawing.
3. Do not allow signage that cannot be understood by patients, families or the community, or that does not meet applicable statutory posting requirements.
4. Do not leave a care area without potable water or electricity; test potable-water quality biochemically at least once in three months and microbiologically at least once a month, collected at the tap.
5. Do not operate without identified backup electricity and water sources available for every critical area.

6. Do not count an alternate source as available if it has not been tested at the defined frequency with documented results.
7. Do not bypass the stop-work authority in section 6 when the trigger conditions are met.
8. Staff who see a FMS.2 rule broken report it the same shift to the Engineering In-Charge or the Medical Superintendent.

5. What we do

5.1 Facilities and space provisions are appropriate to the scope of services

Facilities and space provisions match the scope of services this hospital actually offers, using national or international guidance (including Atomic Energy Regulatory Board guidance where radiation services are in scope).

The Engineering In-Charge and Medical Superintendent keep the current space-vs-services map. A service listed without a matching space is escalated.

5.2 As built and updated drawings are maintained as per statutory requirements.

As-built and updated drawings are maintained as required by the applicable registering authority for this site: site layout, floor plans, floor-wise fire-evacuation plans, and separate civil, electrical, extra-low-voltage, plumbing, heating-ventilation-air-conditioning, piped medical-gas and information-technology drawings.

A named person under the Engineering In-Charge holds the current set. This element does not import a named Act as a checklist for the whole standard.

5.3 There are internal and external sign postings in the organisation in a manner understood by the patient, families and community.

Internal and external sign posting is in a manner patients, families and the community understand (language and/or pictorial; bilingual where this hospital defines). Signs meet applicable statutory posting rules for this site.

The Engineering In-Charge walks the signs quarterly.

5.4 Potable water and electricity are available round the clock

Potable water and electricity are available round the clock. Potable-water quality is tested at the tap: biochemical at least once in three months and microbiological at least once a month, against the current IS 10500. Dialysis reverse-osmosis inlet water is tested for endotoxin every month where dialysis is in scope.

A care area without water or power is a stop-work trigger (section 6).

5.5 Alternate sources for electricity and water are provided as a backup for any failure / shortage.

Alternate sources for electricity and water are provided as backup for any failure or shortage (diesel generator, solar, uninterruptible power supply; bore or tanker or extra tanks as this hospital names). Electric load matches demand. National Building Code is a reference for water quantity, not a paste of every clause.

The Engineering In-Charge owns the backup list. Critical areas have a named continuity action when supply fails.

5.6 The organisation tests the functioning of these alternate sources at a predefined frequency.

The organisation tests these alternate sources at a predefined frequency (default monthly for the diesel generator and a documented water-acceptance test when an emergency source is used). Results are recorded. Refer to FMS.2.d for water quality.

A backup that is never tested does not satisfy this element.

6. Stop-work authority

Do not start or continue clinical care in an area that has no potable water or no electricity, unless the organisation has declared an emergency under the hospital's non-fire emergency plan and the written service-continuity plan for that failure is running.

Stop-work applies to starting or continuing routine care in that area. Immediate life-saving care continues while water or power is restored or the patient is moved.

The person who stops tells the Engineering In-Charge and the Medical Superintendent the same shift. Refusing to run care without water or power is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that FMS.2 is resourced and followed.
Engineering In-Charge	Owns day-to-day implementation and records for this standard.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.
departmental leaders	Run the department-level duties this standard names.

8. Quality monitoring (RCA → CAPA)

The Quality Coordinator audits this policy quarterly. The audit reviews:

- Records for a sample of this standard's objective elements, checked against the What-we-do steps.
- CORE objective elements show no critical gaps in the sample.

- Stop-work events (if any) are logged with outcome.

Root-cause analysis is required when a gap found in this audit remains open beyond 90 days.

This policy is reviewed annually, and sooner after a related facility change, utility failure, equipment recall or fire-plan change.

9. Training and staff acknowledgement

Staff covered by this policy are trained at induction and once a year after that. Training covers the What-we-do steps, non-negotiables and stop-work (if present).

Staff acknowledgement

I have read the Policy on Planned Facilities and Environment-Friendly Measures of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

This policy is distributed to:

- Medical Superintendent
- Engineering In-Charge
- Quality Coordinator
- departmental leaders
- staff covered by FMS.2

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
ELV	Extra Low Voltage
FMS	Facility Management and Safety (NABH Hospitals 6th Edition chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
HVAC	Heating, Ventilation and Air Conditioning
NABH	National Accreditation Board for Hospitals

	and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition FMS.2

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
FMS.2.a	Commitment	Facilities and space provisions are appropriate to the scope of services.	Section 3; 5.1	Engineering In-Charge
FMS.2.b	Commitment	As built and updated drawings are maintained as per statutory requirements.	Section 3; 5.2	Engineering In-Charge
FMS.2.c	CORE	There are internal and external sign postings in the organisation in a manner understood by the patient, families and community.	Section 3; 5.3	Engineering In-Charge
FMS.2.d	CORE	Potable water and electricity are available round the clock.	Section 3; 5.4; Section 6 Stop-work	Engineering In-Charge
FMS.2.e	Commitment	Alternate sources for electricity and water are provided as a backup for any failure / shortage.	Section 3; 5.5	Engineering In-Charge
FMS.2.f	Commitment	The organisation tests the functioning of these alternate sources at a predefined	Section 3; 5.6	Engineering In-Charge

		frequency.		
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13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

FMS.2.a — Facilities and space provisions are appropriate to the scope of services.

- Current space-versus-services map for each department.
- National or international guidance reference used (for example AERB guidance for a radiation area).
- Escalation record for any service listed without matching space.

FMS.2.b — As built and updated drawings are maintained as per statutory requirements.

- Current as-built and updated drawing set: site layout, floor plans, floor-wise evacuation plans, and separate civil, electrical, ELV, plumbing, HVAC, medical-gas and IT drawings.
- Named custodian record for the drawing set.
- Date of last update against a facility change.

FMS.2.c — There are internal and external sign postings in the organisation in a manner understood by the patient, families and community.

- Signage inventory or walk log by area, noting language and/or pictorial form and bilingual coverage.
- Statutory posting-requirement reference applicable to this site.
- Quarterly signage-walk record.

FMS.2.d — Potable water and electricity are available round the clock.

- Potable-water test results: biochemical at least quarterly and microbiological at least monthly, against IS 10500.
- Dialysis reverse-osmosis inlet-water endotoxin test record, where dialysis is in scope.
- Continuity log confirming no unplanned water or power outage in a care area without the hospital's emergency continuity plan activating.

FMS.2.e — Alternate sources for electricity and water are provided as a backup for any failure / shortage.

- Backup-source inventory: diesel generator, solar, uninterruptible power supply, bore/tanker/extra tanks as named.
- Electric-load calculation matching demand.
- Continuity-action record naming each critical area's backup path.

FMS.2.f — The organisation tests the functioning of these alternate sources at a predefined frequency.

- Diesel-generator test log at the defined frequency (default monthly).
- Water-acceptance test record for any occasion an emergency water source was used.
- Corrective-action record for any failed alternate-source test.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Facility Management and Safety, standard FMS.2.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — FMS.2 interpretations.
- Bureau of Indian Standards. IS 10500:2012 (or current version) — Drinking Water Specification. Referenced for potable-water quality testing (biochemical and microbiological) at the point of use.
- Bureau of Indian Standards. National Building Code of India (current version). Referenced for estimating alternate water-supply quantity requirements.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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